



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هبه عبد الله سيد عباس

العنوان :

الحي / المدينة : الهرم

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 19-09-2026

رقم البطاقة : 28508052103184 رقم التليفون : 01151140836

العمر (تاريخ الميلاد) : 05-08-1985 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: هبه عبد الله سيد عباس National ID: 28508052103184	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٠٢٥/٠٥ ٢٨٥٠٨٠٥٢١٠٣١٨٤
 ربة منزل
 انتفى
 مسلمة
 متروجة
 صالح اسماعيل ابراهيم مصطفى
 البطاقة سارية حتى ٢٠٣٢/٠٥/١٧ ١



➔ ١١٥١١٤٠٨٣٦
 ١١٤٥٠٠٥٤٦٤

جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 هبة
 عبدالله سيد عباس
 ٢٢ ش السلام-الكنيسة
 الهرم - الجيزة
 ١٩٨٥/٠٨
 ٢٨٥٠٨٠٥٢١٠٣١٨٤
 LD4613163



Patient's Name: Heba Abdallah Said Abbas
Date: 2026-07-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY

INDICATION:

Left breast lump

FINDINGS:

- Heterogenously dense breast parenchyma which might obscure small masses (ACR c).
- Normal skin, nipple, and areola on both sides
- Normal appearance of retromammary fat on both sides
- Left UOQ focal asymmetry overlapping with irregular small opaque lesion seen at zone b with indistinct margins
- Complementary U/S revealed underlying two irregular solid hypoechoic masses seen at left breast 3 O'clock position 3 cm from the nipple measuring 32x30x20 mm lying 7 mm deep to the skin and the other one measures 14x12 mm lying 5 mm deep to the skin and 7 mm from the first lesion. They both show echogenic rind with positive internal vascularity.
- Right breast shows few rounded circumscribed lucent lesions with thin peripheral outline corresponding to few scattered cysts by ultrasound largest measures 10 mm with low level internal echoes yet no solid component or internal vascularity (oil cysts)
- Right breast also shows retroareolar condensed glandular tissue with no definite underlying lesions by ultrasound
- No detected suspicious solid or cystic masses in the right breast.
- No suspicious left supra or infraclavicular lymph nodes
- No suspicious bilateral axillary lymph nodes, all are oval with preserved fatty hila (reactive).

CONCLUSION:

- Left breast fibroadenosis with UOQ two suspicious lesions as detailed (BIRADS 4c), for biopsy.
- Right breast fibroadenosis with scattered oil cysts (BIRADS 2).
- Further assessment by CESM is recommended.

Best regards,
Dr. Yara Mohamed, MD

Dr. Yara Mohamed, MD
Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name	هبة عبدالله سعيد	Case No.	358211
Sex/Age	Female 41 Y	Patient No.	25705
Registered Date	15/8/2026 02:38:14 PM	Referred By Dr.	Interventional Radiology
Validation Date	08/9/2026 09:39:37 AM	Branch	Gustave Roussy
Reporting Date	19/9/2026 11:01:00 AM	Pathology No.	2518/26

Pathology Report

Clinical Data: Left UOQ breast mass (BIRADS 4c)

Nature of Specimen: Tru-cut biopsy, collected on 10/08/2026, at 1:50 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Multiple fragmented grey, white tissue cores, ranging in size between 0.5 cm & 1 cm long, totally submitted

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), SISH & Ki67 clone (30-9).

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma with focal lobular differentiation.
- **Histologic grade:** 2 (differentiation 3, pleomorphism 2, mitosis 1)
- **DCIS:** Present, low grade, solid
- **Microcalcification:** Absent
- **Angioinvasion:** Absent
- **Perineural invasion:** Absent

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
(Average Her2 copy No.: (3.8), Her2/CEP17 ratio: (2.2))
- Ki67%: 5%.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2 with focal lobular differentiation and low grade solid DCIS.
IPT:

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- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+).
- SISH: Not-amplified.
- Ki67%: 5%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).

- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4

Signature

Prof. Dr. Maha Guimei

Dr. Walaa Ibrahim Ghoneim. MD

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